



- Proton Pump Inhibitor -
Pariet® 10 mg
TABLETS

Pariet® 20 mg
TABLETS

<Rabeprazole sodium preparation>

【DESCRIPTION】

1. Composition

Tablets 10 mg:

Each light yellow, film-coated tablet (enteric coated tablet) contains 10 mg of rabeprazole sodium.

Tablets 20 mg:

Each light yellow, film-coated tablet (enteric coated tablet) contains 20 mg of rabeprazole sodium

2. Product description

Brand name	Dosage form and Identification code	Appearance			Description
		Face	Reverse	Lateral	
PARIET Tablets 10 mg	Film-coated tablets				Light yellow
	E 240	Diameter (mm) 6.7	Weight (mg) 132	Thickness (mm) 3.6	
PARIET Tablets 20 mg	Film-coated tablets				Light yellow
	E 243	Diameter (mm) 7.2	Weight (mg) 163	Thickness (mm) 3.6	

【INDICATIONS】

Healing of Erosive or Ulcerative Gastroesophageal Reflux Disease (GERD)

PARIET® is indicated for short-term (4 to 8 weeks) treatment in the healing and symptomatic relief of erosive or ulcerative gastroesophageal reflux disease (GERD). For those patients who have not healed after 8 weeks of treatment, an additional 8-week course of PARIET® may be considered.

Maintenance of Healing of Erosive or Ulcerative Gastroesophageal Reflux Disease (GERD)

PARIET® is indicated for maintaining healing and reduction in relapse rates of heartburn symptoms in patients with erosive or ulcerative gastroesophageal reflux disease (GERD maintenance).

Healing of Duodenal Ulcers

PARIET® is indicated for short-term (up to four weeks) treatment in the healing and symptomatic relief of duodenal ulcers. Most patients heal within four weeks.

Treatment of Pathological Hypersecretory Conditions, Including Zollinger-Ellison Syndrome

PARIET® is indicated for the long-term treatment of pathological hypersecretory conditions, including Zollinger- Ellison Syndrome.

Symptomatic treatment of moderate to very severe Gastroesophageal reflux disease (symptomatic GERD)

PARIET® is indicated for the symptomatic treatment of moderate to very severe GERD.

Helicobacter pylori Eradication to Reduce the Risk of Duodenal Ulcer Recurrence in Adults

PARIET® in combination with amoxicillin and clarithromycin as a three drug regimen, is indicated for the treatment of patients with *H. pylori* infection and duodenal ulcer disease (active or history within the past 5 years) to eradicate *H. pylori*. Eradication of *H. pylori* has been shown to reduce the risk of duodenal ulcer recurrence [see Clinical Studies and Dosage and Administration].

In patients who fail therapy, susceptibility testing should be done. If resistance to clarithromycin is demonstrated or susceptibility testing is not possible, alternative antimicrobial therapy should be instituted [see Clinical Pharmacology].

【DOSAGE AND ADMINISTRATION】

The usual adult dose for oral use is 10 mg of sodium rabeprazole administered once daily. However, the dosage may be increased up to 20 mg orally once daily depending on the severity of symptoms.

Adults/elderly:

Healing of Erosive or Ulcerative Gastro-Oesophageal Reflux Disease (GERD):

The recommended oral dose for this condition is 20 mg to be taken once daily for four to eight weeks. Doses of 10 mg or 20 mg twice daily may be administered orally to reflux esophagitis patients for a further 8 weeks when proton pump inhibitor treatment is ineffective. However, a dose of 20 mg twice daily should only be administered to patients with severe mucosa injury.

Gastro-Oesophageal Reflux Disease Long-term Management (GERD Maintenance):

For long-term management, a maintenance dose of PARIET 20 mg or 10 mg once daily can be used depending upon patient response.

For the maintenance therapy of reflux esophagitis when proton pump inhibitor treatment is ineffective, dose of 10 mg twice daily may be administered orally.

Zollinger-Ellison Syndrome:

The recommended adult starting dose is 60 mg once a day. The dose may be titrated upwards to 120 mg/day based on individual patient needs. Single daily doses up to 100 mg/day may be given. 120 mg dose may require divided doses, 60 mg twice daily. Treatment should continue for as long as clinically indicated.

Symptomatic treatment of moderate to very severe gastro-oesophageal reflux disease (symptomatic GERD):

10 mg once daily in patients without oesophagitis. If symptom control has not been

achieved during four weeks, the patient should be further investigated. Once symptoms have resolved, subsequent symptom control can be achieved using an on-demand regimen taking 10 mg once daily when needed.

Helicobacter pylori Eradication to Reduce the Risk of Duodenal Ulcer Recurrence in Adults

THREE DRUG REGIMEN ^a		
PARIET	20 mg	Twice Daily for 7 Days
Amoxicillin	1000 mg	Twice Daily for 7 Days
Clarithromycin	500 mg	Twice Daily for 7 Days

All three medications should be taken twice daily with the morning and evening meals.

^a It is important that patients comply with the full 7-day regimen

For indications requiring once daily treatment PARIET tablets should be taken in the morning, before eating; and although neither the time of day nor food intake was shown to have any effect on rabeprazole sodium activity, this regimen will facilitate treatment compliance.

Patients should be cautioned that the PARIET tablets should not be chewed or crushed, but should be swallowed whole.

Renal and hepatic impairment: No dosage adjustment is necessary for patients with renal or hepatic impairment.

Children:

PARIET is not recommended for use in children, as there is no experience of its use in this group.

<Precaution>

The patient should be carefully observed during administration and the cumulative dose should be kept to a minimum for the particular symptoms being treated.

【PRECAUTIONS】

1. Contraindication

Patients with known hypersensitivity to rabeprazole sodium, or to any excipient used in the formulation. PARIET is contra-indicated in pregnancy and during breast feeding.

2. Careful Administration (PARIET should be administered with care in the following patients.)

- (1) Patients with a history of drug hypersensitivity
- (2) Patients with hepatic function disorder
[Psychoneurotic adverse reactions have been reported in patients with liver cirrhosis. (See “Adverse Reactions” section.)]
- (3) Elderly patients (See “Use in the Elderly” section.)

3. Important Precautions

PARIET can be administered at a dose of 20 mg once daily in the case of severe, recurrent and intractable conditions.

4. Drug Interactions

It has been reported that the hepatic enzyme cytochromes P450 2C19 (CYP2C19) and 3A4 (CYP3A4) are involved in the metabolism of PARIET. [See “PHARMACOKINETICS” section.]

Precautions for coadministration (PARIET should be administered with care when coadministered with the following drugs.)

Drugs	Signs, Symptoms, and Treatment	Mechanism and Risk Factors
Digoxin	Blood concentration of digoxin may increase.	Absorption of digoxin is promoted due to increasing gastric pH.
Phenytoin	It has been reported that an analogue compound (omeprazole) delays metabolism and excretion of phenytoin.	
Antacid containing aluminum hydroxide gel/magnesium hydroxide	It has been reported that the mean area under the plasma sodium rebeprazole concentration-time curve (AUC) decreases by 8 and 6% after concomitant administration of PARIET and antacid, and administration of PARIET at 1 hr after the antacid, respectively, compared to the administration of PARIET alone.	
Methotrexate	Blood concentration of methotrexate may increase. In cases where high-dose methotrexate is administered, a temporary withdrawal of PARIET may be considered.	The mechanism remains unclear.

5. Adverse Reactions

Adverse reactions were reported in 22 of 1,244 patients (1.77%), and abnormal laboratory test values were reported in 82 (6.59%). (At the time of approval)

(1) Clinically significant adverse reactions

1) Shock

It has been reported that anaphylactic reactions or shock occurred with analogue compounds (omeprazole and lansoprazole). If any such abnormality is observed, treatment should be discontinued and appropriate measures taken.

2) Agranulocytosis, thrombocytopenia and pancytopenia

Agranulocytosis (incidence unknown), thrombocytopenia (5%> ≥ 0.1%)

or pancytopenia (incidence unknown) may occur. If any such abnormality is observed, treatment should be discontinued and appropriate measures taken. It has been reported that hemolytic anemia occurred with analogous compounds (omeprazole and lansoprazole).

3) Hepatic function disorders

Hepatic function disorders (5%> ≥10%) or jaundice (incidence unknown) may occur. If any such abnormality is observed, treatment should be discontinued and appropriate measures taken.

4) Interstitial pneumonia

Interstitial pneumonia (incidence unknown) may occur. If symptoms such as fever, coughing, dyspnoea and abnormal lung sounds (crepitations) occur, thoracic radiography or other examination should be performed immediately. Administration should be discontinued, and appropriate measures should be taken, such as treatment with cortical steroid hormones.

5) Visual disturbance

Visual disturbance has been reported with an analogue compound (omeprazole).

6) Interstitial Nephritis

7) *Clostridium Difficile* associated diarrhea

8) Vitamin B12 Deficiency

(2) Other adverse reactions

	5% > ≥0.1%	<0.1%	Incidence unknown
Hypersensitivity ^{note 1)}	Rash and urticaria	Itching	
Hematologic ^{note 2)}	Erythrocytopenia, leukopenia, leukocytosis, eosinophilia, neutrophilia and lymphopenia		Anamia
Hepatic ^{note 3)}	Elevation of AST (GOT), ALT (GPT), AL-P, γ-GTP, LDH and total bilirubin		
Cardiovascular		Palpitations	
Gastrointestinal	Constipation, diarrhea and feeling of enlarged abdomen, fundic gland polyps (benign)	Nausea, lower abdominal pain and heavy feeling of stomach	Microscopic colitis
Psychoneurologic ^{note 4)}	Headache	Dizziness, sleepiness, weakness in the extremities, hypoaesthesia, decreased grip strength, impaired tongue movement and light headedness	
Musculoskeletal disorders		Fracture of the hip, wrist or spine	
Skin and subcutaneous tissue disorders			Subacute lupus erythematosus
Others	Elevation of total cholesterol, triglycerides and BUN, and proteinuria, and increase in blood TSH	Malaise and fever, edema, blurred vision, alopecia, numbness and elevation of CK (CPK)	Blurred vision and shifting vision, hyperammonemia, hypomagnesaemia and gynaecomastia

Note 1) In the event of such symptoms, treatment should be discontinued.

2) It is advisable to conduct hematological tests periodically during administration of PARIET. If any abnormality is observed, appropriate measures, such as discontinuation of the medication, should be taken.

3) It is advisable to conduct biochemical tests periodically during the administration of PARIET. If any abnormality is observed, appropriate measures, such as discontinuation of the medication, should be taken.

4) Weakness in the extremities, hypoaesthesia, decreased grip strength, impaired tongue movement, light headedness, delirium, coma and disorientation have been reported in a patient with liver cirrhosis. Also, confusion and lethargy have been reported in a patient with liver cirrhosis who had a history of hepatic encephalopathy.

(3) Post-Marketing Adverse Events

Additional adverse events reported from worldwide marketing experience with sodium rabeprazole are: sudden death, coma and hyperammonemia, jaundice, rhabdomyolysis, disorientation and delirium, bullous and other drug eruptions of the skin, interstitial pneumonia, interstitial nephritis, and TSH elevations. In most instances, the relationship to sodium rabeprazole was unclear. In addition, agranulocytosis, hemolytic anemia, leukopenia, pancytopenia and thrombocytopenia have been reported. Increases in prothrombin time/INR in patients treated with concomitant warfarin have been reported.

6. Use in the Elderly

PARIET is metabolized mainly in the liver. Since the physiological functions of the liver are often reduced in the elderly, they are more likely to experience adverse reactions. Therefore, if adverse reactions such as gastrointestinal symptoms (see “Adverse Reactions” section) occur, it is advisable to take measures such as instituting a drug-free interval with careful supervision.

7. Use during Pregnancy, Delivery or Lactation

There are no data on the safety of rabeprazole in human pregnancy. Reproduction studies performed in rats and rabbits have revealed no evidence of impaired fertility or harm to the foetus due to rabeprazole sodium, although low foeto-placental transfer occurs in rats. PARIET is contraindicated during pregnancy.

[Fetotoxicity (delayed ossification in rats, weight loss and delayed ossification in rabbits) has been reported with PARIET in animal studies (400 mg/kg p.o. in rats, 30 mg/kg i.v. in rabbits).]

It is not known whether rabeprazole sodium is excreted in human breast milk. No studies in lactating women have been performed. Rabeprazole sodium is however excreted in rat mammary secretions. Therefore PARIET should not be used during breast feeding.

[In an animal study (in rats), it has been reported that PARIET is excreted in breast milk.]

8. Pediatric Use

PARIET is not recommended for use in children, as there is no experience of its use in this group.

Effects on ability to drive and use machines?

Based on the pharmacodynamic properties and the adverse events profile, it is unlikely that PARIET would cause an impairment of driving performance or compromise the ability to use machinery. If however, alertness is impaired due to somnolence, it is recommended that driving and operating complex machinery be avoided.

9. Precautions concerning Use

(1) Regular Surveillance

Patients on proton pump inhibitor treatment (particularly those treated for long term) should be kept under regular surveillance.

(2) Administration

Since PARIET is an enteric coated tablet, patients should be instructed not to chew or crush the tablet, but swallow it whole.

(3) Caution in handing over drug

For drugs that are dispensed in a press-through package (PTP), instruct the patient to remove the drug from the package prior to use. [It has been reported that, if the PTP sheet is swallowed, the sharp corners of the sheet may puncture the esophageal mucosa, causing perforation and resulting in serious complications such as mediastinitis.]

10. Other Precautions

(1) It has been reported that in a carcinogenicity study in which 5mg/kg/day or greater of sodium rabeprazole was administered orally to rats for 2 years, carcinoids were observed in the stomachs of female rats.

(2) Increases in thyroid weight and blood thyroxine levels have been reported in animal studies (rats, oral administration of 25mg/kg/day or greater). Therefore, thyroid function should be carefully monitored during the administration of PARIET.

(3) Benign gastric polyp has been reported during long-term administration of PARIET.

(4) Fracture

Proton pump inhibitors, especially if used in high doses and over long durations (>1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in older people or in presence of other recognised risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10–40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of vitamin D and calcium.

(5) *Clostridium Difficile* Diarrhea

Published observational studies suggest that PPI therapy may be associated with an increased risk of *Clostridium difficile* associated diarrhea, especially in hospitalized patients. This diagnosis should be considered for diarrhea that does not improve.

Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated.

(6) Interference with laboratory test

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours.

To avoid this interference, PARIET treatment should be stopped for at least 5 days before CgA measurements (see Pharmacology). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

(7) Subacute cutaneous lupus erythematosus (SCLE)

Proton pump inhibitors are associated with very infrequent cases of SCLE. If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping PARIET. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors.

(8) Hypomagnesaemia

Severe hypomagnesaemia has been reported in patients treated with proton pump inhibitors like PARIET for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI.

For patients expected to be on prolonged treatment or who take PPI with digoxin or drugs that may cause hypomagnesaemia (e.g., diuretics), health care professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment.

(9) Vitamin B12 Deficiency

Daily treatment with any acid-suppressing medications over a long period of time (e.g., longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B-12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy have been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency are observed.

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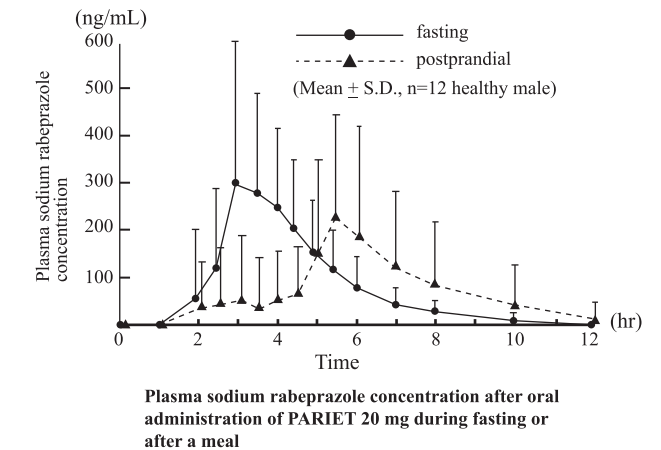
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[PHARMACOKINETICS]

1. Blood concentrations

The changes over time in the mean plasma sodium rabeprazole concentration when administered orally to healthy adult male volunteers at a dose of 20 mg during fasting or after a meal are shown in the figure below. Mean values of pharmacokinetic parameters determined for individual subjects during fasting and postprandial administration of PARIET are presented in the table below. T_{max} was prolonged by 1.7 hr after postprandial administration compared to administration during fasting, and inter-individual variations in absorption were observed.



Effect of food on pharmacokinetic parameters

Dose	C _{max} (ng/mL)	t _{max} (hr)	AUC (ng·hr/mL)	t _{1/2} (hr)
Fasting	437 ± 237	3.6 ± 0.9	937 ± 617	1.49 ± 0.68
Postprandial	453 ± 138	5.3 ± 1.4	901 ± 544	1.07 ± 0.47

(Mean ± S.D., n = 12)

The following table shows the mean values of pharmacokinetic parameters when sodium rabeprazole was administered orally to healthy adult male volunteers at doses of 10 mg and 20 mg during fasting.

Pharmacokinetic parameters after single oral administration of PARIET to healthy adult male volunteers

Dose	C _{max} (ng/mL)	t _{max} (hr)	AUC (ng·hr/mL)	t _{1/2} (hr)
10 mg	247 ± 24	3.8 ± 0.5	440 ± 24	0.85 ± 0.04
20 mg	406 ± 64	3.1 ± 0.2	809 ± 186	1.02 ± 0.16

(Mean ± S.E., n = 6)

2. Metabolism

When PARIET was administered orally to healthy adult male volunteers at single doses of 10 mg and 20 mg, the main metabolite recognized in plasma was its thioether-form product produced by non-enzymatic reduction reaction. Other metabolites were the demethylated products due to demethylation which involving the hepatic enzymatic metabolism of cytochrome P450 2C19 (CYP2C19), and the sulfone-form products due to sulfonation involving 3A4 (CYP3A4).

3. Urinary excretion

No unchanged drug was detected in the urine of healthy adult male volunteers up to 24 hr after oral administration of 20 mg of sodium rabeprazole, and about 29-40% of the dose was excreted in the urine as the carboxylic acid form and its glucuronide, and about 13-19% of the dose was present as the mercapturate conjugated-form.

[CLINICAL STUDIES]

Clinical efficacy

The results of open-labeled and double-blind clinical trials conducted with PARIET in patients with gastric ulcer, duodenal ulcer, reflux esophagitis and anastomotic ulcer are summarized in the following table.

Disease	Endoscopic healing rate
Gastric ulcer	95.2% (401/421)
Duodenal ulcer	98.1% (364/371)
Reflux esophagitis	90.9% (50/55)
Anastomotic ulcer	83.3% (10/12)

The overall improvement rate in 2 patients with Zollinger-Ellison syndrome was 100%.

It was demonstrated in clinical pharmacology studies that the increase in gastric pH was greater in the 20 mg group than in the 10 mg group, and the usefulness of PARIET in the treatment of intractable ulcer has been demonstrated using a dose of 20 mg once daily.

In addition, the clinical usefulness of PARIET in the treatment of gastric and duodenal ulcers has been demonstrated in double-blind clinical trials.

The following table shows endoscopic non-recurrence rate at week 52 in the Maintenance Period (Central Evaluation) with PARIET for patients with reflux esophagitis resistive to treatment ^{note} with standard doses of proton pump inhibitors.

	10 mg once daily	10 mg twice daily	Difference in the non-recurrence rate between groups ^{a)} (95% CI) <i>P</i> -value ^{b)}
Non-recurrence rate at week 52	44.8% (73/163)	73.9% (119/161)	29.1(18.9, 39.3) <i>P</i> <0.001

a) 10 mg twice daily group – 10 mg once daily group b) χ^2 test

Note) Uncured cases after more than 8 weeks treatment with 10 mg or 20 mg of sodium rabeprazole daily, 30 mg of lansoprazole daily, 20 mg of omeprazole daily or 20 mg of esomeprazole daily, and recurrent cases during maintenance therapy.

Helicobacter pylori Eradication in Patients with Peptic Ulcer Disease or Symptomatic Non-Ulcer Disease in Adults

The U.S. multicenter study was a double-blind, parallel-group comparison of rabeprazole, amoxicillin, and clarithromycin for 3, 7, or 10 days vs. omeprazole, amoxicillin and clarithromycin for 10 days. Therapy consisted of rabeprazole 20 mg twice daily, amoxicillin 1000 mg twice daily, and clarithromycin 500 mg twice daily (RAC) or omeprazole 20 mg twice daily, amoxicillin 1000 mg twice daily, and clarithromycin 500 mg twice daily (OAC). Patients with *H. pylori* infection were stratified in a 1:1 ratio for those with peptic ulcer disease (active or a history of ulcer in the past five years) [PUD] and those who were symptomatic but without peptic ulcer disease [NPUD], as determined by upper gastrointestinal endoscopy. The overall *H. pylori* eradication rates, defined as negative 13C-UBT for *H. pylori* ≥ 6 weeks from the end of the treatment are shown in the following table. The eradication rates in the 7 day and 10 day RAC regimens were found to be similar to 10 day OAC regimen using either the Intent-to-Treat (ITT) or Per-Protocol (PP) populations. Eradication rates in the RAC 3 day regimen were inferior to the other regimens.

HELICOBACTER PYLORI ERADICATION AT ≥ 6 WEEKS AFTER THE END OF TREATMENT

	Treatment Group Percent (%) of Patients Cured (Number of Patients)		Difference (RAC – OAC) [95% Confidence Interval]
	7-day RAC ^a	10-day OAC	
Per Protocol ^b	84.3% (N = 166)	81.6% (N = 179)	2.8 [-5.2, 10.7]
Intent-to-Treat ^c	77.3% (N = 194)	73.3% (N = 206)	4.0 [-4.4, 12.5]
	10-day RAC ^a	10-day OAC	
Per Protocol ^b	86.0% (N = 171)	81.6% (N = 179)	4.4 [-3.3, 12.1]
Intent-to-Treat ^c	78.1% (N = 196)	73.3% (N = 206)	4.8 [-3.6, 13.2]
	3-day RAC	10-day OAC	
Per Protocol ^b	29.9% (N = 167)	81.6% (N = 179)	-51.6 [-60.6, -42.6]
Intent-to-Treat ^c	27.3% (N = 187)	73.3% (N = 206)	-46.0 [-54.8, -37.2]

- a The 95% confidence intervals for the difference in eradication rates for 7 day RAC minus 10 day RAC are (-9.3, 6.0) in the PP population and (-9.0, 7.5) in the ITT population.
- b Patients were included in the analysis if they had *H. pylori* infection documented at baseline, defined as a positive 13C-UBT plus rapid urease test or culture and were not protocol violators. Patients who dropped out of the study due to an adverse event related to the study drug were included in the evaluable analysis as failures of therapy.
- c Patients were included in the analysis if they had documented *H. pylori* infection at baseline as defined above and took at least one dose of study medication. All dropouts were included as failures of therapy.

[PHARMACOLOGY]

1. Mechanism of action

PARIET transforms to the activated form (sulfenamid form) at parietal cells in acidic conditions, and acts by modification of SH-groups of the proton pump (H⁺, K⁺-ATPase) causing inhibition of enzyme activity and consequents acid secretion suppression. It is believed that the recovery of enzyme activity is mainly due to drug elimination from the active site, or that glutathione may be involved in the elimination of the active drug. The involvement of glutathione in recovery of enzyme activity is also suspected.

2. Action in humans

- (1) Inhibition of gastric acid secretion
- When PARIET was administered to healthy adult male volunteers at 10 mg or 20 mg once a day, gastrin-stimulated acid output was significantly decreased compared with the 1st day of administration. The mean percentages of acid output reduction compared with the day before starting administration on day 1 and on day 7 were, at 10 mg once a day, 73 % and 80%, and at 20 mg once a day, 88-89 % and 99%, respectively.
- (2) Increase reaction of intragastric pH
- When PARIET was administered to healthy adult male volunteers at 10 mg or 20 mg once a day, intragastric pH was significantly increased. The percentage of holding times above pH 4 and pH 3 in 24 hr on day 4 after treatment with 10 mg

was 73% and 80% respectively, and for treatment with 20 mg was 78% and 83%, respectively.

3. Action in animals

- (1) Inhibition of H⁺, K⁺-ATPase (*in vitro*)
- Sodium rabeprazole strongly inhibits H⁺, K⁺-ATPase in preparations made from pig gastric mucosa.
- (2) Inhibition of gastric acid secretion
- 1) Sodium rabeprazole inhibits gastric acid secretion stimulated by dibutyl cyclic-AMP in isolated rabbit gastric glands (*in vitro*).
- 2) Sodium rabeprazole exhibits strong inhibition of gastric acid secretion stimulated by histamine or pentagastrin in chronic gastric fistula dogs as well as basal gastric acid secretion and histamine-stimulated gastric acid secretion in rats. Compared to other proton pump inhibitors, the reversal of the antisecretory effect is more rapid with sodium rabeprazole and the increase in blood gastrin levels is less in dogs and rats.
- (3) Antiulcer action
- In rats, sodium rabeprazole demonstrated a strong antiulcer action against various experimental ulcers and therapeutic activity in experimental gastric mucosal lesions (induced by cold restraint stress, water immersion stress, pyloric ligation, cysteamine or ethanol-hydrochloride).

4. Adjunctive effect on *Helicobacter pylori* Eradication

Susceptibility testing of *H. pylori* isolates was performed for amoxicillin and clarithromycin using agar dilution methodology, and minimum inhibitory concentrations (MICs) were determined. Standardized susceptibility test procedures require the use of laboratory control microorganisms to control the technical aspects of the laboratory procedures. Incidence of Antibiotic-Resistant Organisms Among Clinical Isolates

Pretreatment Resistance: Clarithromycin pretreatment resistance rate (MIC ≥ 1 mcg/mL) to *H. pylori* was 9% (51/560) at baseline in all treatment groups combined. A total of > 99% (558/560) of patients had *H. pylori* isolates which were considered to be susceptible (MIC ≤ 0.25 mcg/mL) to amoxicillin at baseline. Two patients had baseline *H. pylori* isolates with an amoxicillin MIC of 0.5 mcg/mL.

CLARITHROMYCIN SUSCEPTIBILITY TEST RESULTS AND CLINICAL/ BACTERIOLOGIC OUTCOMES⁸ FOR A THREE DRUG REGIMEN (RABEPAZOLE 20 MG TWICE DAILY, AMOXICILLIN 1000 MG TWICE DAILY, AND CLARITHROMYCIN 500 MG TWICE DAILY FOR 7 OR 10 DAYS)

Days of RAC Therapy	Clarithromycin Pretreatment Results	Total Number	<i>H. pylori</i> Negative (Eradicated)	<i>H. pylori</i> Positive (Persistent) Post-Treatment Susceptibility Results			
				S ^b	I ^b	R ^b	No MIC
7	Susceptible ^b	129	103	2	0	1	23
7	Intermediate ^b	0	0	0	0	0	0
7	Resistant ^b	16	5	2	1	4	4
10	Susceptible ^b	133	111	3	1	2	16
10	Intermediate ^b	0	0	0	0	0	0
10	Resistant ^b	9	1	0	0	5	3

- a Includes only patients with pretreatment and post-treatment clarithromycin susceptibility test results.
- b Susceptible (S) MIC ≤ 0.25 mcg/mL, Intermediate (I) MIC = 0.5 mcg/mL, Resistant (R) MIC ≥ 1 mcg/mL

Patients with persistent *H. pylori* infection following rabeprazole, amoxicillin, and clarithromycin therapy will likely have clarithromycin resistant clinical isolates. Therefore, clarithromycin susceptibility testing should be done when possible. If resistance to clarithromycin is demonstrated or susceptibility testing is not possible, alternative antimicrobial therapy should be instituted.

Amoxicillin Susceptibility Test Results and Clinical/Bacteriological Outcomes: In the U.S. multicenter study, a total of > 99% (558/560) of patients had *H. pylori* isolates which were considered to be susceptible (MIC ≤ 0.25 mcg/mL) to amoxicillin at baseline. The other 2 patients had baseline *H. pylori* isolates with an amoxicillin MIC of 0.5 mcg/mL, and both isolates were clarithromycin-resistant at baseline; in one case the *H. pylori* was eradicated. In the 7 and 10 day treatment groups 75% (107/145) and 79% (112/142), respectively, of the patients who had pretreatment amoxicillin susceptible MICs (≤ 0.25 mcg/mL) were eradicated of *H. pylori*. No patients developed amoxicillin-resistant *H. pylori* during therapy.

During treatment with antisecretory medicinal products, serum gastrin increases in response to the decreased acid secretion. Also, CgA increases due to decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours.

Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is to allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range.

[PHYSICOCHEMISTRY]

Nonproprietary name: Sodium Rabeprazole(JAN)

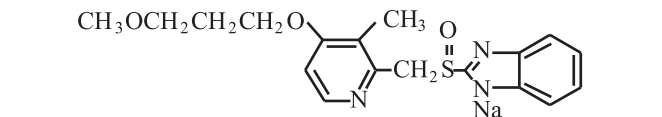
Chemical name:

(±)-sodium2-[[4-(3-methoxypropoxy)-3-methylpyridin-2-yl]methylsulfinyl]-1*H*-benzimidazole

Molecular formula: C₁₈H₂₀N₃O₃SSNa

Molecular weight: 381.43

Structural formula:



Description:

Sodium rabeprazole occurs as a white to pale yellowish white powder. It is odorless. It is very soluble in water and in methanol, freely soluble in dehydrated ethanol and in ethyl acetate, and practically insoluble in ether and in hexane. It shows no optical rotation. It is hygroscopic. Melting point: 225°C (with decomposition)

Partition coefficient: about 214 (pH7.0, water: octanol)

[STORAGE AND HANDLING]

Storage:

PARIET[®] should be stored at room temperature not exceeding 30° C and protected from moisture after unsealing.

Expiration date:

PARIET[®] should be used before the expiration date indicated on the package (It is also recommended that PARIET[®] be used as soon as possible after unsealing.).

[PACKAGING]

PARIET[®] tablets 10 mg:

Boxes of 14 and 280 in press-through packages

PARIET[®] tablets 20 mg:

Boxes of 14 and 280 in press-through packages

[NOTE]

Date of revision:

August 2020

Manufactured by:

Bushu Pharmaceuticals Ltd. Misato Factory
950, Hiroki, Ohaza, Misato-machi, Kodama-gun, Saitama-ken, Japan

Repacked by:

INTERTHAI PHARMACEUTICAL MANUFACTURING LTD.
1899 PHAHOLYOTHIN ROAD, LADYAO, CHATUCHAK, BANGKOK 10900 THAILAND

Product registration holder:

Eisai (Malaysia) Sdn. Bhd.
Unit 701D, Level 7, Tower D, Uptown 5
No. 5, Jalan SS21/39, Damansara Uptown
47400 Petaling Jaya
Selangor, Malaysia





Client : Eisai	Date : 22/8/2020	References Color :  Black	 Digital Proof	Client Approved by :
Product : Leaflet Pariet (MS)				Date :
Code : 112400167 P8/20	Dimension : 382x290 mm.			

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